

Paragraph

[LOCATION]

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Name: [NAME]

Study Date: [DATE]

ID: 215753

[HOSPITAL]: 76891

MRI AND MRA EXAMINATION OF THE [HOSPITAL]:

TECHNIQUE:

- Sagittal T2WI
- Axial T2 and [HOSPITAL]
- Coronal T1WI
- 3D TOF MRA.

FINDINGS:

I- MRI:

· A well-defined intra-axial right thalamic hematoma seen displaying isointense signal in [LOCATION] and dark signal in T2 [HOSPITAL]'s denoting acute blood products signal ([HOSPITAL]). It measures about 24 x 27 mm in dimensions and showing intra-ventricular extension reaching the fourth ventricle.

" Left posterior temporo-parietal cortical/subcortical old infarction seen with [HOSPITAL] like signal and associated underlying gliotic tissues.

- Bilateral corona radiata and centrum semiovale old lacunar infarcts seen.

· Prominent cortical brain sulci with widened extra-axial [HOSPITAL] spaces and widened sylvian fissures.

· Voluminous supratentorial ventricular system with no hydrocephalic changes or deformity.

* <NRP>-ventricular deep white matter sheets and patches of high signal intensity in [HOSPITAL] seen denoting small vessels leukoencephalopathy.

- The cerebellum and brain stem are also normal.

· No extra axial collections.

[HOSPITAL]

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Name: AHMED ADEL [HOSPITAL]

Study Date: [DATE]

ID: 11891

[HOSPITAL]: 50605

MRA EXAMINATION OF THE [HOSPITAL]

TECHNIQUE:

- TOF MRA

FINDINGS:

11- [HOSPITAL]

· Absent horizontal segment of right [HOSPITAL]. (NORMAL VARIANT)

* Patent normal caliber common carotid and both anterior and middle cerebral arteries. No evidence of stenotic segments, vessel occlusion or vascular malformation.

* Patent average caliber of the basilar and both posterior cerebral arteries with no evidence of vessel occlusion or stenotic segments.

OPINION:

- Unremarkable [HOSPITAL] studies of the brain.

Dr [NAME],[LOCATION]

[HOSPITAL]: [DATE]

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